

THE PROS AND CONS OF WHERE TO HAVE YOUR BABY

Remember that it's never too late to change your mind and if your circumstances change you may have to rethink your decision. This table sets out some of the points to consider when you are deciding where to have your baby.



Hospital



Instant specialized medical attention if necessary.

- » All forms of pain relief available.
- » All technical gadgets and gizmos to react quickly in a changing labor situation.
- » Baby unit (NICU) usually available if your baby needs medical attention immediately after birth.



Lack of continuous care

- » The doctor on call who supports you in labor may not be your main OB/GYN.
- » There may be changes of shifts during your labor, so you may see several hospital nurses before you have your baby.
- » Count on spending a night or two after your baby is born.



Birthing center



Midwife-led care in a low-tech environment.

- » Although the unit is independently run, full medical care is close by.
- » More relaxed birthing environment.
- » More opportunity to let your labor run its course without intervention.
- » Lower rates of assisted birth (use of forceps or vacuum extractor).
- » May lead to a shorter labor.
- » Fewer cesarean sections.
- » Greater chance that your birth partner can stay overnight.



- » **40 percent of women giving birth to their first baby** have to go into the hospital anyway (and about 10 percent for second or subsequent babies), although there is more immediate access to full medical care.



Home



Continuous care.

- » Your chosen midwife is likely to be the person who supports you in labor.
- » You won't have to endure traveling during labor.
- » Statistically reduced risk of assisted birth.
- » You're at home with your baby right from the start.



40 percent of women giving birth to their first baby

- » have to go to the hospital anyway (and about 10 percent for second or subsequent babies).
- » No access to pain relief.
- » Your birth partner may not be allowed to come in the ambulance with you if you have to go to the hospital.
- » Home births are opposed by the American Medical Association and the American College of Gynecologists and Obstetricians because of the potential for complications, even in low-risk pregnancies.

- Will someone show me how to change a diaper, sponge bathe, and bathe the baby?
- What happens if I decide to bottle-feed—do I have to bring the bottles and formula with me?
- Can we buy snacks and drinks here during labor, or should we bring them with us?
- What facilities do you have for playing music in the birthing room?

What to ask at the hospital

- Will I be moved to a ward after I've had the baby?
- Is an anesthesiologist always available if I want an epidural?

- What are the visiting hours and do they apply to my partner? What are the visiting hours for siblings?
- Are there any single rooms and, if so, how are they allocated? Do we have to pay for them?

What to ask at a birthing center

- How quickly could I get to a hospital if I needed to, and would I need to go in an ambulance?
- For what reasons would I need to go to a hospital and which hospital would I go to?
- Is a medical doctor able to come to the center in an emergency?



Did you know...

In 1900, almost all babies in the US were born at home. Gradually, births moved to hospitals and the percentage of home births fell: 44% by 1940 and 1% by 1969. Although that number has been rising since 2004, it's still below 1% per year. A home birth isn't recommended by the American Medical Association because there could be complications, even in healthy women who could need to be transferred to a hospital.